A/PROF CHAMARA BASNAYAKE

MBBS, PhD, FRACP

Gastroenterologist & Endoscopist

PN 421926PK

FOCUS GASTROENTEROLOGY

27 May 2026

Dr Shanthini Sathiyalngam
Brandon Park Medical Centre

Dear Dr Sathiyalngam,

Re: Mrs Marie Manuelpillai
DOB: 10/01/1955 Phone: 0422 271 365
Address: 1 Boronia Drive, GLEN WAVERLEY, VIC, 3150
Email: marie@mpillai.com

Summary

- **Intestinal metaplasia in the distal stomach for surveillance gastroscopy in two years.**
- **Benign colonic polyp with lymphocytic infiltration; lymphoma excluded on immunohistochemistry.**
- **Small hiatus hernia and associated symptoms of eructation.**
- **Continuation of prucalopride 1mg for symptom management.**
- **No action required by you.**

I had the pleasure of conducting a telehealth review with Marie following her recent gastroscopy and colonoscopy.

The colonoscopy revealed a polyp which has been confirmed as benign. Interestingly, the pathologist noted a significant lymphocytic infiltrate within the polyp, which necessitated further staining and analysis to exclude the possibility of lymphoma. I am pleased to report that these additional investigations were negative, confirming the findings are entirely benign. The remainder of the colonic biopsies were normal, and as this was not a precancerous lesion, Marie does not require routine surveillance colonoscopy.

Regarding her gastroscopy, we identified a small hiatus hernia and some inflammation in the antrum. More importantly, biopsies confirmed the presence of intestinal metaplasia. While I had suspected this during her previous gastroscopy, the biopsies at that time were normal. This current finding confirms a low-risk subtype involving only a small area of the stomach.

Impression and Plan I have reassured Marie that while intestinal metaplasia requires monitoring due to a very small risk of malignant transformation, her current risk profile is low. I have recommended a surveillance gastroscopy in two years' time. Her symptoms of frequent burping are likely related to the small hiatus hernia, which predisposes her to reflux, though she does not find this particularly distressing.

In terms of her medications, we discussed her ongoing use of prucalopride. I have provided a prescription for the 2mg dose. We discussed the possibility of her trialling a lower dose or weaning the medication to see if her symptoms remain controlled. We also reviewed her previous response to rifaximin; as she did not find it beneficial previously, I do not believe there is any utility in repeating a course. I have suggested a follow-up review in 12 months, and I have advised her to contact me sooner should she develop any red-flag symptoms

All appointments and enquires

Suite 201, 100 Victoria Parade
East Melbourne VIC 3002
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Email: office@focusgastro.com.au

Endoscopy locations

Epworth Freemasons Day Proc. Centre
320 Victoria Pde, East Melbourne, 3002
Hobson Healthcare Sydenham
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St Vincent's Private Hospital
59 Victoria Pde, Fitzroy, 3065

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such as melaena or haematochezia.

Yours sincerely,

A/Prof Chamara Basnayake
Gastroenterologist

Investigation Results:

FINAL ANATOMICAL PATHOLOGY AAnat

Lab #: 2026B012271

SUPPLEMENTARY REPORT 20/05/2026

Additional three levels performed and no adenomatous change is seen. Negative for dysplasia.

SUPPLEMENTARY REPORT 19/06/2026

Specimen 3 - A panel of immunohistochemical stains performed on specimen 3. The majority of lymphocytes are T cells and are highlighted by CD3 and CD5 immunostains. Some CD20+ cells (B cells) are also highlighted. BCL2 highlights most lymphocytes except follicles (similar to CD3). BCL6 and CD10 highlight germinal centres. CD21 shows follicular dendritic meshwork with no significant irregularity or expansion. The features would be in keeping with a prominent intramucosal lymphoid aggregate with reactive change. No adenomatous changeis seen.A few additional levels will be examined. A second supplementary report to follow.

Specimen 2 - H pylori immunostain performed on specimen 2 and is negative. The diagnosis remains unchanged.

Pathologist: Dr Simin Daneshvar

CLINICAL NOTES

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Antrum =? IM.

Body = N.

Random colon.

Caecal polyp.

MACROSCOPIC DESCRIPTION

1. Specimen labelled "Gastric Body bx".

A mucosal fragment measuring mm. A1.

2. Specimen labelled "Gastric Antrum bx".

Two mucosal fragments measuring 2-4 mm. A1.

3. Specimen labelled "Caecal Polyp".

A tan polypoid fragment measuring 17 x 6 x 1 mm.

Resection margin inked blue. 5 TS. A1.

4. Specimen labelled "Random Colonic bx".

Two mucosal fragments measuring 1-4 mm. A1

MICROSCOPIC DESCRIPTION

1. The sections consist of specialised gastric mucosa with preserved glandular architecture. There are scattered chronic inflammatory cells within the lamina propria. Intestinal metaplasia is not a feature. No neutrophils, Helicobacter-like organisms, or dysplasia identified.

2. There are fragments of antral type gastric mucosa with pits showing reactive change and tortuous appearance. There is lamina propria fibrosis with patchy moderate chronic inflammation with foci of mild neutrophilic inflammation. There are foci of intestinal metaplasia. No obvious Helicobacter organisms are seen. Negative for dysplasia or granuloma.

3. There is a strip of colonic mucosa partially showing lymphoid aggregates mostly composed of small sized lymphocytes with scattered

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histiocytes. Elsewhere no adenomatous change is seen. Negative for dysplasia.

4. There is colonic mucosa with relatively preserved crypt architecture. There is focal mild increase in the numbers of chronic inflammatory cells within the lamina propria. Negative for neutrophilic inflammation, granuloma or dysplasia. Features are not diagnostic of microscopic colitis.

PATHOLOGIST'S OPINION

1. **Gastric body biopsy - Within normal morphological limits.**
2. **Gastric antrum biopsy - Moderate chronic focally active gastritis with reactive change and intestinal metaplasia.**

COMMENT: H.pylori immunostain will be performed and a supplementary report issued when results become available.

3. **Caecal polyp - Colonic mucosa with a prominent aggregate of lymphocytes. A panel immunohistochemical stains to follow.**
4. **Random colon biopsy - Mild non-specific change.**

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